

Health Insurance Claims Analysis

Executive Summary

Period: 2024-01 to 2024-07
Scope: multi-country underwriter portfolio

Total Claims	Total Paid (USD)	Total Premium (USD)	Loss Ratio
12	\$5,323	\$8,104	65.7%

Executive Summary

The portfolio's health claims performance indicates a moderate loss ratio of 65.67%, suggesting manageable risk exposure relative to premiums collected. Total claims volume remains low at 12, with a total paid amount of \$5,322.53 against \$8,104.42 in premiums. Hamad General Hospital accounts for 54.84% of claims, signaling concentrated provider utilization. Essential Hypertension is the leading diagnosis, highlighting potential chronic

Key Findings

- The loss ratio of 65.67% reflects a balanced claims-to-premium relationship, i current portfolio sustainability.
- Hamad General Hospital's 54.84% share of claims suggests provider concentr warrant network optimization review.
- Essential Hypertension as the top diagnosis underscores the need for targeter disease management strategies.

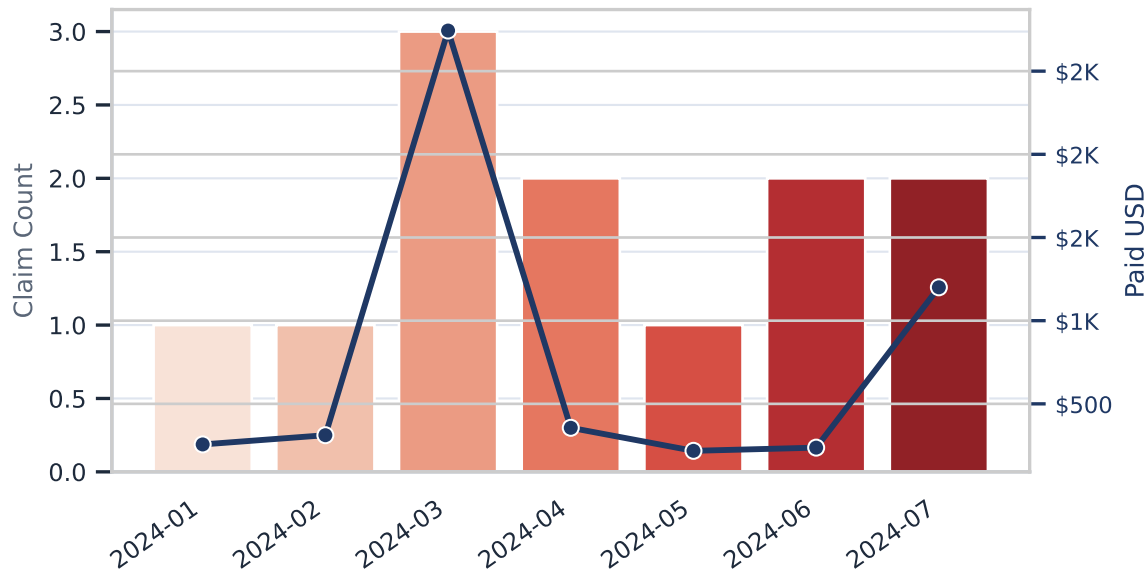
Top Recommendations

- Evaluate provider network agreements to ensure competitive pricing and mitigate risks associated with provider concentration.
- Implement chronic disease management programs focused on hypertension to reduce long-term claim costs and improve member health outcomes.
- Monitor claims trends quarterly to identify emerging risks and adjust underwriting or wellness initiatives accordingly.

Health Insurance Claims Analysis

Claims Overview

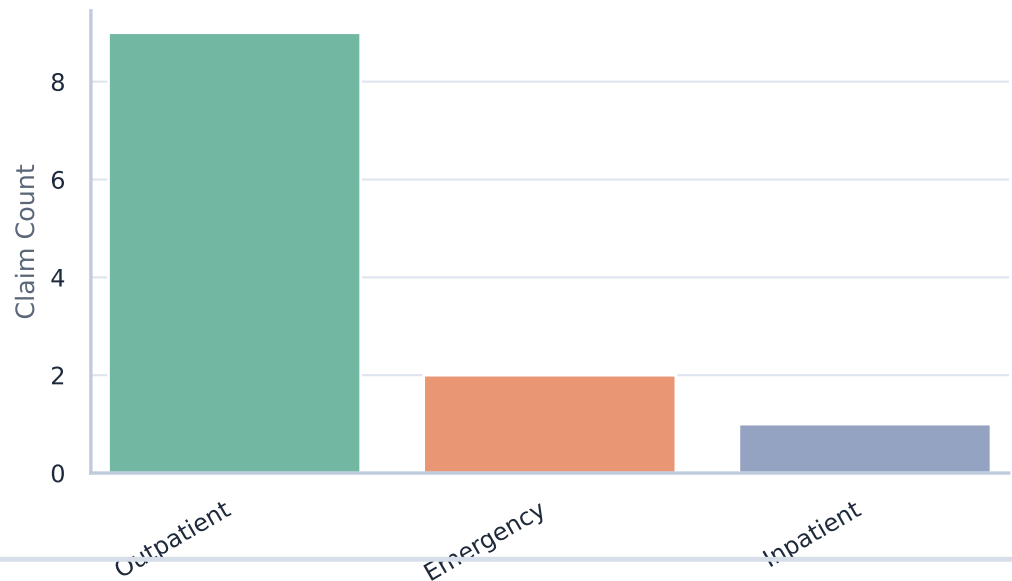
Monthly Claim Volume



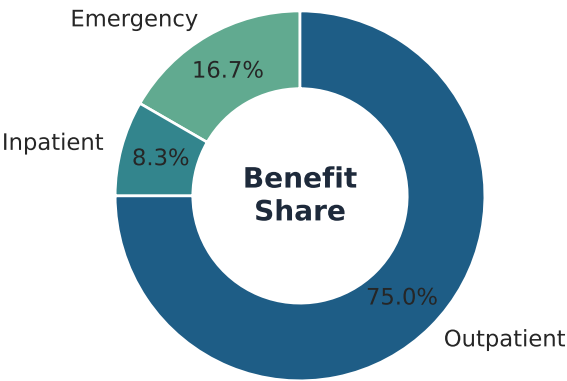
Quarterly Summary

Quarter	Total Claims	Total Paid Usd	Avg Cost Per Claim
Q1	5	3,312	662
Q2	5	810	162
Q3	2	1,201	600

Benefit Mix by Claim Count



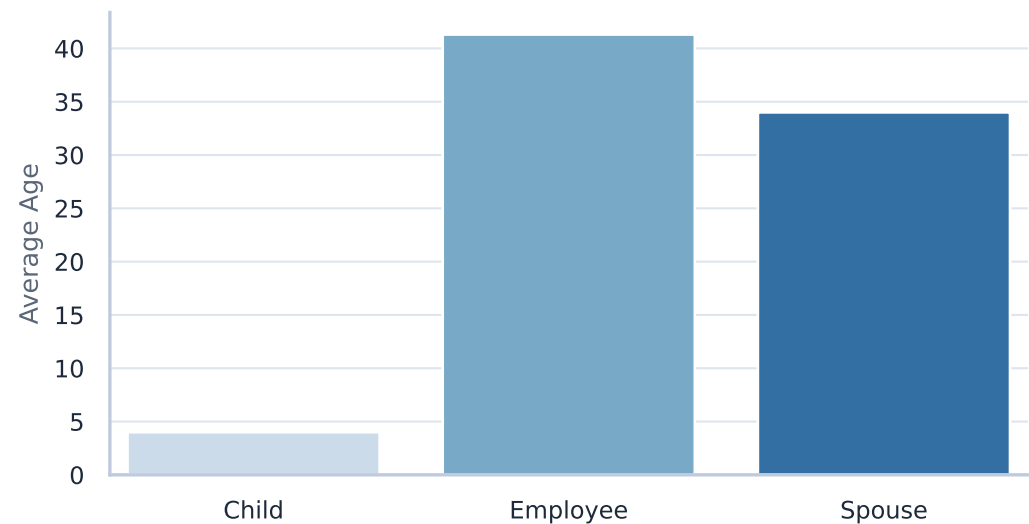
Benefit Distribution



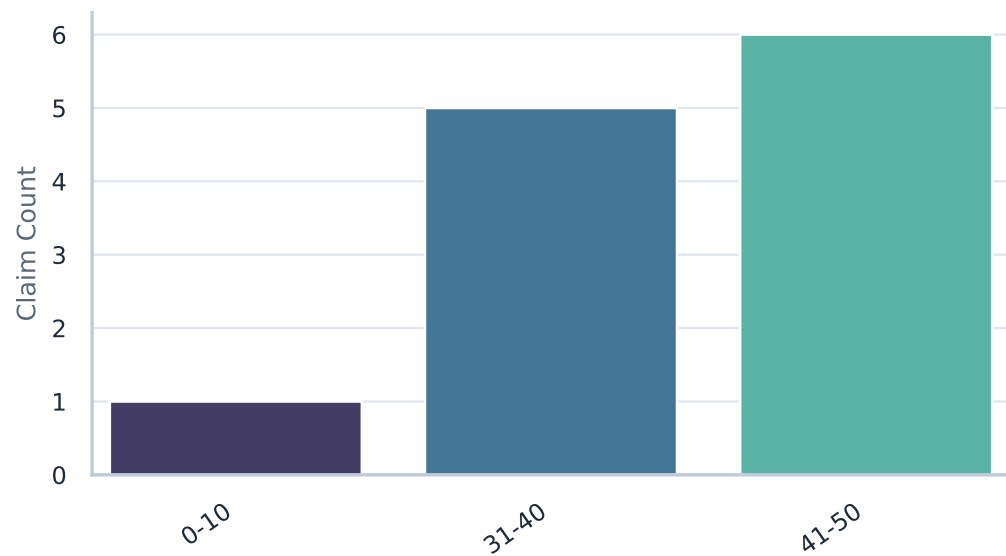
Health Insurance Claims Analysis

Demographics and Cost Profile

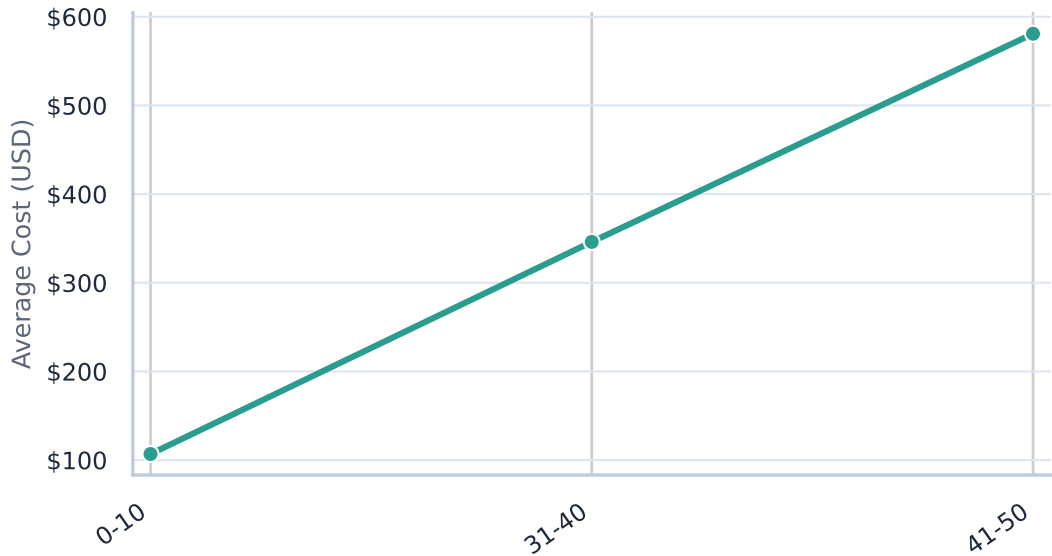
Average Age by Relationship



Claims by Age Group

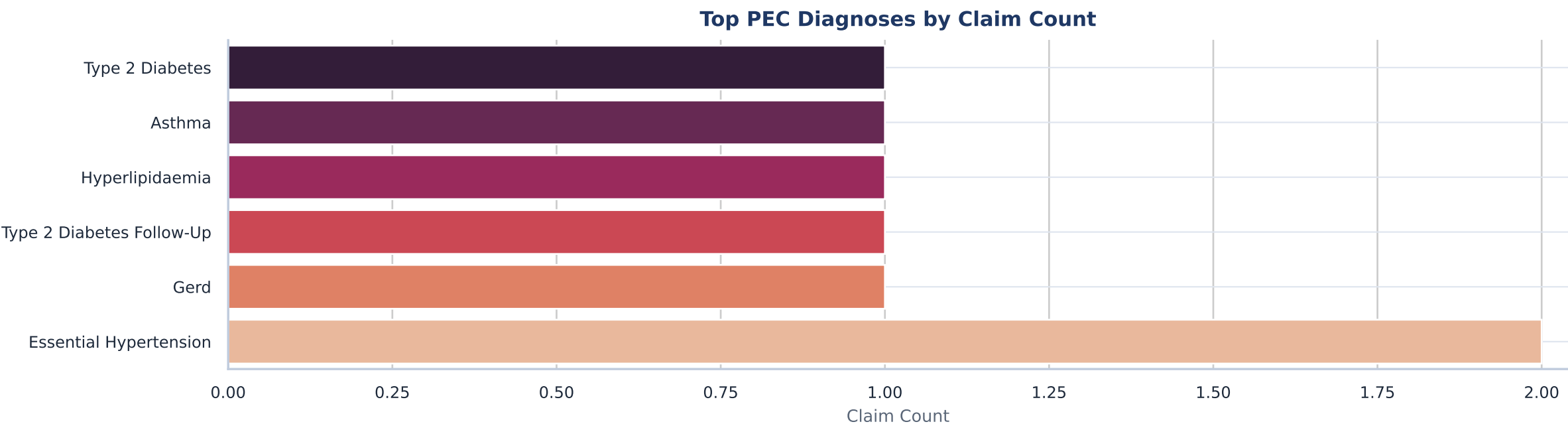


Average Cost by Age Group



Demographic Insights

- Highest claim-volume age group: 41-50.
- Highest average-cost age group: 41-50.
- Top provider concentration currently sits at 54.8%.
- Current portfolio loss ratio is 65.7%.



PEC Summary Table

Icd10 Code	Diagnosis Description	Total Claims	Total Paid Usd
I10	Essential Hypertension	2	434
E11	Type 2 Diabetes	1	312
J45	Asthma	1	188
E78	Hyperlipidaemia	1	182
E11	Type 2 Diabetes Follow-Up	1	178
K21	Gerd	1	113

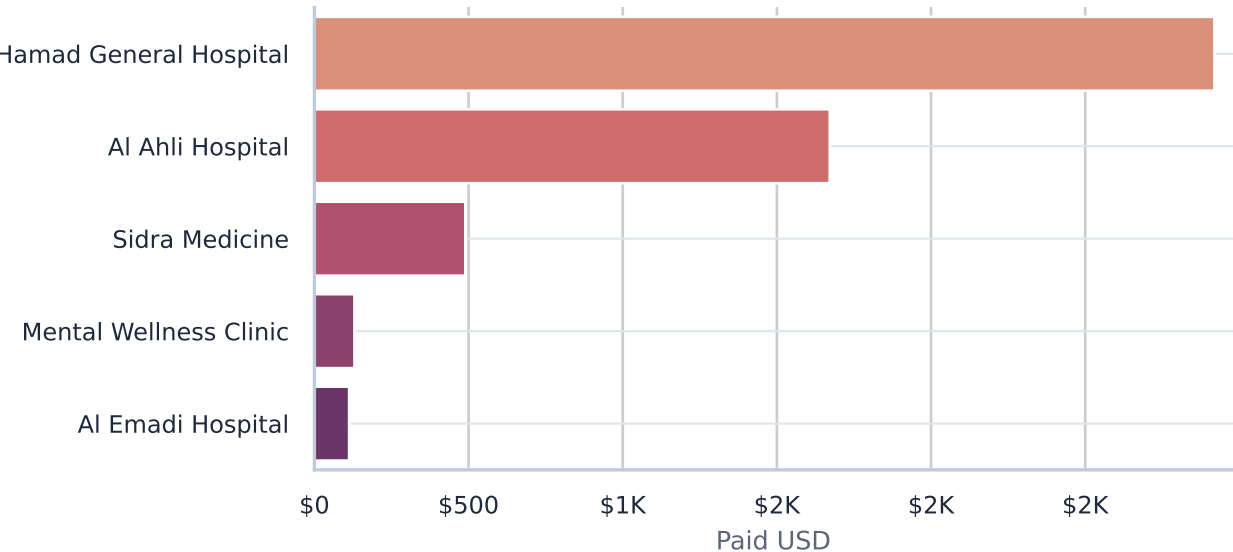
Clinical Insights

- High utilization of Hamad General Hospital may indicate member preference or limited provider options, requiring further analysis.
- Essential Hypertension prevalence suggests a need for proactive interventions, such as medication adherence programs or lifestyle coaching.
- Low claims volume provides an opportunity to establish predictive analytics for early identification of high-

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Provider and Country Performance

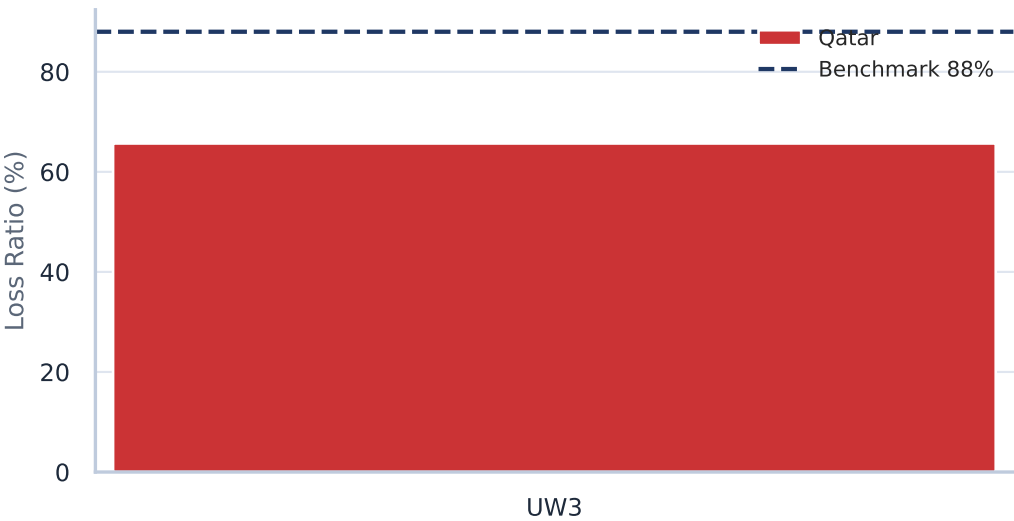
Top Providers by Paid Amount



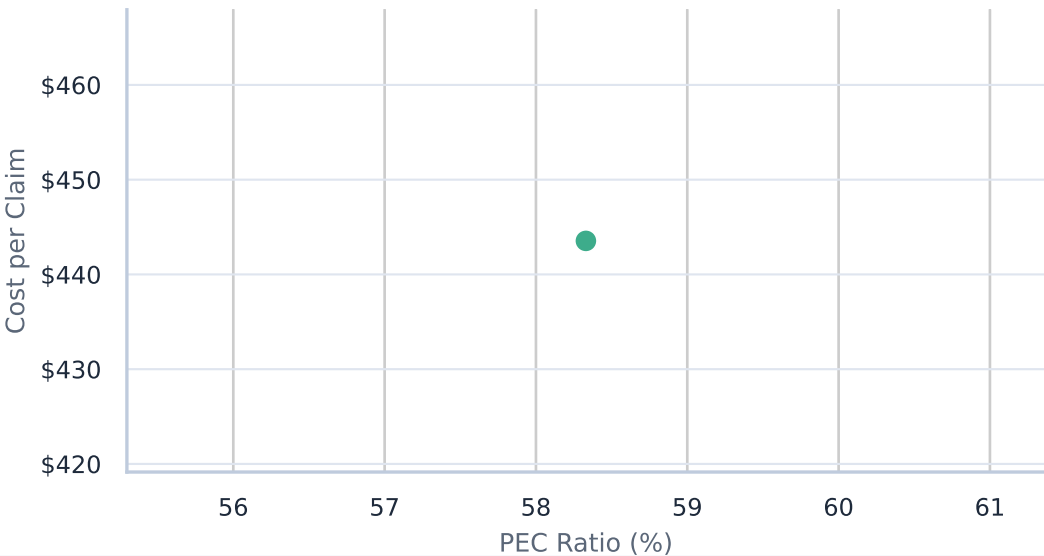
Provider Summary

Provider Name	Total Claims	Total Paid USD	Paid Share (%)	Avg Cost Per Claim
Hamad General Hos	4	2,919	54.8%	730
Al Ahli Hospital	4	1,671	31.4%	418
Sidra Medicine	2	490	9.2%	245
Mental Wellness Cl	1	130	2.4%	130
Al Emadi Hospita	1	113	2.1%	113

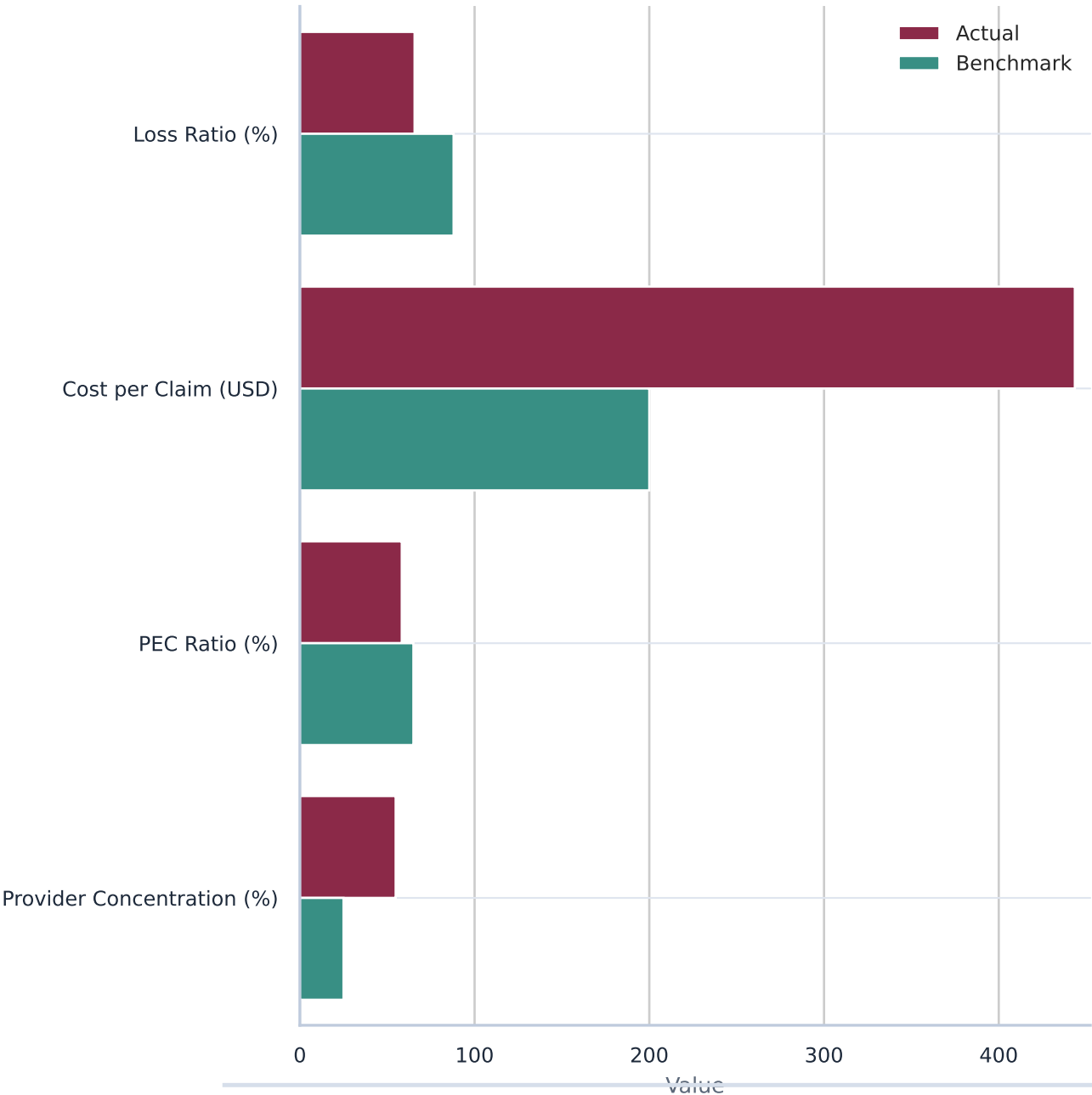
Underwriter Loss Ratio Comparison



PEC Burden vs Cost per Claim



Actual vs Benchmark



Handoff Notes

- PDF pack includes 8 pages with KPI, trend, demographic, and PEC analyses.
- Power BI-ready tables are generated in CSV format and can be connected directly.
- Narrative sections can run in fallback mode or use Azure OpenAI for richer wording.
- Use this report layout as a deterministic baseline for future underwriter uploads.

Action plan focused on chronic utilization, provider leverage, and country-specific controls.

1. Chronic Disease Management Programme

Business case:

PEC/Chronic conditions currently drive 58.3% of portfolio claims. Hypertension and diabetes contribute 4 diagnosed episodes.

Action:

Run annual HbA1c, blood pressure, and lipid screening with a regional wellness partner; use telemedicine follow-up for confirmed chronic members.

Est. saving:

USD 113 to 169 per year from a 10% episode reduction.

2. Provider Network Renegotiation

Business case:

Top provider concentration is 54.8% of paid amount, creating renewal pricing risk and negotiation dependency.

Action:

Request itemized billing audits for top-spend providers and negotiate a portfolio-level volume discount across countries.

Est. saving:

USD 175 to 263 per year with a 6%-9% discount range.

3. Qatar Portfolio Pre-Authorization Tightening

Business case:

Qatar loss ratio is currently 65.7% and top two high-cost claims contribute 65.0% of Qatar paid value.

Action:

Mandate pre-authorization for inpatient admissions above USD 1,500 in Qatar and add second-opinion review for oncology treatment plans.

Est. saving:

Risk mitigation action to prevent loss-ratio drift above renewal tolerance bands, especially where case concentration is high.

4. Class C / Bronze Network Consolidation

Business case:

Class C/Bronze segment in Qatar averages USD 121 per claim and can be managed with tighter provider panels.

Action:

Review Bronze networks and consolidate to fewer preferred providers (e.g., Mental Wellness Clinic + Al Emadi Hospital) after HR and employee communication sign-off.

Est. saving:

USD 29 to 44 per year via consolidation and admin efficiency.

Full KPI Summary

Metric	Value	Benchmark	Status
Total Claims	12	-	-
Total Paid (USD)	USD 5,323	-	-
Loss Ratio	65.7%	85-95%	Below range
Cost Per Claim (USD)	USD 444	USD 150-250	Above range
PEC/Chronic Ratio	58.3%	< 65%	Within range
Inpatient %	8.3%	25-35%	Below range
Avg Age (Employee)	41.3 yrs	30-42 yrs	Within range
Oncology Cases	0 claims	< 5 per period	Within range
Maternity Cases	2 claims	-	-

Methodology and Currency Conversion

- Loss Ratio = Total Paid Amount (USD) / Gross Written Premium (USD) x 100.
- UW1 (Oman): Amounts in Baissa/Fils are converted to OMR then to USD as per pipeline rules.
- UW2 (Saudi Arabia): Amounts are treated as USD where already normalized in source.
- UW3 (Qatar): Amounts in QAR are converted to USD using configured conversion factors.

Disclaimer

- Prepared from underwriter-provided claim records for the selected reporting period.
- Benchmarks are directional and should be validated during renewal discussions.
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